TriClip™ G5 Steerable Guide Catheter

Rx ONLY

Instructions for Use

WARNING: Use of the TriClip™ G5 Steerable Guide Catheter (Guide) should be restricted to those physicians trained to perform invasive endovascular and transcatheter procedures.

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1.0 DEVICE DESCRIPTION

The TriClip™ G5 Steerable Guide Catheter (Guide) provides a conduit to access the right atrium. The distal curve is optimal for achieving height above tricuspid valve plane. The distal shaft is steered by the use of a handle and knobs (S/L and +/-) to position the implant on the tricuspid valve. The Guide tip is radiopaque. The Guide is fitted with a valve to provide hemostasis during introduction and / or exchange of ancillary devices (e.g., catheters) that have a maximum diameter of 5.2 mm (.204”). A flush port is provided on the valve housing for aspiration, fluid infusion, and blood sampling via a three way stopcock attachment. The Guide includes a Dilator that is tapered at the distal tip and includes an echogenic feature for visualization under imaging. The Dilator has an internal lumen designed to accept ancillary devices that have a maximum diameter of 0.9 mm (0.035”) (e.g., needles or guidewires). The distal 65 cm of the Steerable Guide Catheter with the exception of the distal soft tip is coated with hydrophilic coating to provide a lubricated surface when hydrated with sterile saline for ease of insertion.

Table 1: TriClip™ G5 Steerable Guide Catheter Dimensions

Component	*Dimension*
TriClip™ G5 Steerable Guide Catheter	
Working Length	*78.0 cm*
Catheter Proximal Shaft Outer Diameter	*8.4 mm (25 Fr)*
Catheter Distal Shaft / Tip Outer Diameter	*7.7 mm (23 Fr)*
Dilator	
Working Length	*122.0 cm*
Shaft Inner Diameter	*1.0 mm (3 Fr)*
Shaft Outer Diameter	*5.4 mm (16 Fr)*
Distal Tip Outer Diameter	*1.5 mm (4 Fr)*

2.0 INDICATION FOR USE

The TriClip™ G5 Steerable Guide Catheter is used for introducing various cardiovascular catheters into the right side of the heart.

3.0 CONTRAINDICATIONS

- Intolerance, including allergy or untreatable hypersensitivity, to procedural anticoagulation.
- Active endocarditis or other active infection of the tricuspid valve.

4.0 WARNINGS

- Read all instructions carefully. Failure to follow these instructions, warnings and precautions may lead to device damage, user injury or patient injury including:
  - Failure to deliver the TriClip™ G5 Implant to the intended site
  - Difficulty or failure to retrieve TriClip™ G5 System components
- Use universal precautions for biohazards and sharps while handling the TriClip™ G5 Steerable Guide Catheter to avoid user injury.
- Failure to prepare the device as stated in these instructions and failure to handle the device with care might result in damage to the device coating, which may lead to additional intervention or serious adverse events.

5.0 PRECAUTIONS

- Use the TriClip™ G5 Steerable Guide Catheter with sterile techniques using standard imaging techniques (e.g., fluoroscopy and transesophageal echocardiography).
- Echocardiographic images should be carefully assessed to ensure they are of adequate quality to allow successful implantation of the TriClip™ G5 Implant using the TriClip™ G5 Steerable Guide Catheter.
- The TriClip™ G5 Procedure using the TriClip™ G5 Steerable Guide Catheter should be considered with caution in patients with rheumatic disease who have significant leaflet thickening and small annular dimensions considering the risk of iatrogenic tricuspid stenosis.
- Patients with pre-existing cardiac leads should be assessed to ensure placement of the TriClip™ Implant is possible using the TriClip™ G5 Steerable Guide Catheter.

6.0 SPECIAL FACILITIES

Facilities using the TriClip™ G5 Steerable Guide Catheter need to maintain a plan for treating the patient if cardiovascular surgical intervention is required.

7.0 HOW SUPPLIED

7.1 Contents:

One (1) TriClip™ G5 Steerable Guide Catheter, one (1) Dilator, and one (1) Silicone Pad.

7.2 Sterile:

The TriClip™ G5 Steerable Guide Catheter is sterilized with ethylene oxide gas and is provided in a tray with lid, in a sealed pouch. Parts of the device that are in either direct or indirect contact with circulating blood are non-pyrogenic.

8.0 STORAGE

Handle with care. Store in original packaging. Keep dry. Keep away from sunlight.

9.0 ADDITIONAL REQUIRED EQUIPMENT NOT INCLUDED

Exchange length supportive guidewire
High pressure three way stopcocks (2)
Arterial high-pressure extension tubing (1)
50–60 cc syringes with luer fitting (2)
Heparinized sterile saline solution
Sterile Basin

10.0 PATIENT PREPARATION

WARNING: When placing a TriClip™ G5 Implant using the TriClip™ G5 Steerable Guide Catheter, refer to the TriClip™ G5 System Instructions For Use.

WARNING: Patient selection should be performed by a multi-disciplinary heart team. Refer to the TriClip™ G5 System Instructions For Use for patient selection and potential adverse events information.

Prepare the patient per institution's standard practice for endovascular and transcatheter procedures. Prepare the patient for invasive hemodynamic monitoring.

11.0 DEVICE PREPARATION BEFORE USE

The preparation is most easily accomplished with the aid of an assistant.

Inspect all product prior to use; do not use if the device or packaging is damaged. Note the product "Use By" date specified on the package; do not use if the "Use By" date has passed. These devices are intended for single-use only; do not reuse.

WARNING: DO NOT use the TriClip™ G5 Steerable Guide after the "Use By" date stated on the package label, and never reuse or re-sterilize the Steerable Guide Catheter. Use of expired, reused, or re-sterilized devices may result in infection, malfunction of the device and other serious injury or death.

WARNING: Always inspect the TriClip™ G5 Steerable Guide Catheter and its packaging to verify no damage has occurred as a result of shipping and handling and that the sterile barrier has not been compromised. DO NOT use the device if damage is detected. Use of product with a compromised sterile barrier may result in infection. Use of damaged product may result in patient injury.

12.0 TRICLIP™ G5 STEERABLE GUIDE CATHETER PREPARATION

WARNING: All lumens contain air when shipped. Use proper de-airing techniques before and during use to minimize the risk of air embolism.

12.1 Remove the Guide from sterile packaging and place in the sterile field.

12.2 Carefully remove the white Guide tip shape retainer and transparent protective tubing from the Guide tip.

12.3 Inspect the Guide and Dilator to verify they are undamaged.

WARNING: DO NOT use if damage is detected. Use of damaged product may result in air embolism, vascular and / or cardiac injury.

12.4 Remove the sterile package containing the Silicone Pad from the Guide tray.

12.5 Fill a basin with heparinized saline.

12.6 Flush and de-air the Guide and Dilator with heparinized saline:

- 12.6.1 Connect 3-way stopcocks to the Guide and Dilator flush ports, de-air the Dilator, then close the stopcock and the Rotating Hemostatic Valve.
- 12.6.2 Hydrate 5-10 cm of the distal end of the Dilator with heparinized saline and insert the Dilator approximately 10 cm into Guide then remove.
- 12.6.3 Connect high pressure tubing and a 50-60 cc syringe filled with heparinized saline to the Guide flush port.
- 12.6.4 De-air the Guide.
  - 12.6.4.1 With the tip raised, displace all air from the Guide while tapping along the length of the catheter shaft.
  - 12.6.4.2 Cover the Guide tip with finger once heparinized saline exits the Guide.
  - 12.6.4.3 Close the Guide stopcock.

12.7 Submerge the Guide tip in the basin of heparinized saline.

12.8 While the Guide tip is submerged in the basin of heparinized saline, remove finger from Guide tip and check the Guide valve for leaks by raising the handle to a vertical position for a minimum of 30 seconds.

12.9 Hydrate 5-10 cm of the distal end of the Dilator with heparinized saline.

12.10 Cover the Guide tip with finger and insert the Dilator into the Guide while Guide tip remains submerged in the basin of heparinized saline.

- 12.10.1 While advancing the Dilator, continually watch for air in the Guide Hemostasis Valve housing. If needed, remove finger from Guide tip and aspirate while assuring the Guide tip is submerged.
- 12.10.2 Remove finger from tip of Guide when the Dilator tip approaches the Guide tip.
- 12.10.3 Position the Dilator to create a smooth transition.

WARNING: Confirm a smooth transition between the Dilator and the tip of the Guide to minimize the risk of vascular and / or cardiovascular injury.

13.0 ACCESS TO THE TRICUSPID VALVE

NOTE: This is a suggested sequence for the procedure. Variations may be used based upon patient anatomy.

13.1 Access the RA to accommodate the Guide tip using transvenous techniques and equipment.

13.2 Heparinize the patient.

WARNING: Failure to administer heparin may result in thrombus formation.

13.3 Carefully place an exchange length supportive guidewire in the superior vena cava. Dilate the subcutaneous tissue and femoral vein to accommodate the Guide shaft using standard dilation technique.

14.0 TRICLIP™ G5 STEERABLE GUIDE CATHETER INSERTION

CAUTION: Always use pressure monitoring, echocardiography and fluoroscopy for guidance and observation during use of the TriClip™ G5 Steerable Guide Catheter.

WARNING: Always use a careful, deliberate, and iterative approach to positioning the TriClip™ G5 Steerable Guide Catheter. It is recommended to make multiple small adjustments rather than single large adjustments. Large adjustments may result in vascular and / or cardiac injury.

14.1 Rotate the +/- Knob in the "-" direction until the Guide curve is substantially straightened.

14.2 Wet the surface of the Guide shaft with sterile saline.

14.3 Insert the Guide-Dilator assembly over the stationary guidewire into the femoral vein.

WARNING: DO NOT use excessive force to advance or manipulate the Guide-Dilator assembly. If resistance is encountered, use echocardiography and / or fluoroscopy to assess before proceeding. Use of excessive force may result in arrhythmias, vascular and / or cardiac injury.

14.4 Advance the Guide-Dilator assembly to the RA maintaining the Guide in a straightened position.

14.5 Adjust Guide torque to position the tip away from adjacent tissues.

14.6 Place the Silicone Pad on the sterile drape over the Lift. Place the Stabilizer onto the Silicone Pad, such that the Stabilizer Guide Dock is underneath the Guide Attach.

14.7 Rotate the Guide Attach independently from the Guide as necessary to align the Guide Attach pins with the Stabilizer Guide Dock.

14.8 Insert the Guide Attach into the Stabilizer Guide Dock.

CAUTION: Ensure that the Stabilizer is secure on the lift to prevent any undesirable movement.

14.9 Retract the Dilator approximately 5 cm into the Guide, leaving the guide wire in the superior vena cava.

14.10 Retract the guidewire into the tip of the Dilator. Remove the Dilator and guidewire while gently aspirating the Guide (starting when the Dilator is approximately halfway retracted into the Guide, approximately 40 cm) using a 50–60 cc syringe. Cover Guide Hemostasis Valve with finger upon Dilator removal.

NOTE: Avoid contacting tissue or creating a vacuum in the Guide lumen. If necessary, position the Guide handle below the level of the RA to allow blood to fill the Guide lumen.

WARNING: **DO NOT create a vacuum while removing the Dilator from the Guide; air may enter the lumen of the Guide which may result in air embolism.**

WARNING: **Failure to fully retract guidewire into the Dilator may result in air embolism.**

15.0 GUIDE REMOVAL AFTER DEVICE USE

15.1 Straighten the Guide with the +/- Knob and turn the S/L Knob to neutral.

15.2 Carefully retract the Guide tip into the RA / Inferior Vena Cava junction. The Guide may be straightened further with the +/- Knob if desired.

15.3 Retract the Guide towards the femoral vein then remove the Guide while providing hemostasis.

16.0 DISPOSAL

After use, these devices and packaging should be appropriately classified for disposal, e.g., biohazard, sharps, non-hazardous waste etc., and carefully disposed of in compliance with facility procedures and applicable laws and regulations.

17.0 GLOSSARY OF ABBREVIATIONS

Guide:	TriClip™ G5 Steerable Guide Catheter	RV:	Right Ventricle
RA:	Right Atrium	RO:	Radiopaque

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GRAPHICAL SYMBOLS FOR MEDICAL DEVICE LABELING

Source reference for recognized symbols retained at the manufacturer in the Medical Device File.

LOT	Batch code		Do not resterilize
REF	Catalogue number		Do not re-use
	Use-by date		Non-pyrogenic
STERILE EO	Sterilized using ethylene oxide		Do not use if package is damaged and consult instructions for use
	Single sterile barrier system		Manufacturer
	Consult instructions for use or consult electronic instructions for use		Date of manufacture
	Refer to instruction manual/booklet (Symbol Color: Blue)		Packaging unit
MR	MR Conditional		Keep away from sunlight
UDI	Unique device identifier		Keep dry
MD	Medical device	Rx ONLY	CAUTION: Federal law restricts this device to sale by or on the order of a physician.